

Provider Time Burden from Food & Beverage Payments

SRHS vs. National, Regional, and State Benchmarks (2018-2025)

RHP Pharma Influence Analysis | Spartanburg Regional Health System | July 13, 2026

Gabriel Cabrera, Data Scientist 1

Executive Summary

Food & Beverage (F&B) payments are the highest-volume, lowest-dollar category of industry payment in CMS Open Payments data - typically a meal or snack tied to a sales-rep visit or program. Regardless of dollar value, each F&B event represents an interaction that consumes provider time. This report sets aside dollar value and drug-payment causality (covered in other reports in this project) and asks a narrower, standalone question over the full eight-year history available: how much industry F&B contact do SRHS providers experience, purely by volume of events, compared to national and regional (NC/SC) peers - and is that volume concentrated in specific specialties or manufacturers, and stable or changing over time?

SRHS providers who received at least one F&B payment between 2018 and 2025 average 124.7 events over the full window (15.6 per year) - 2.37x the national rate, 2.75x the North Carolina rate, and 2.29x the South Carolina rate. All four groups dipped sharply in 2020 (COVID-19 disruption to in-person rep visits) and recovered afterward, but the recovery paths diverge: National, NC, and SC each plateaued at roughly their pre-2020 baseline from 2022 onward, while SRHS kept climbing past its own pre-COVID peak, reaching an all-time high in 2025.

As a result, the SRHS-to-National ratio has widened from about 1.4x in 2021 to 1.91x in 2025. Family Medicine, Nurse Practitioner (general and Family), Obstetrics & Gynecology, Orthopedic Surgery, Dermatology, and Nurse Anesthetists (CRNA) are now the specialties combining large, credible SRHS samples (n=22 to n=229) with meaningfully elevated rates (2.08x-2.84x national peers in the same specialty).

AbbVie is confirmed as the dominant manufacturer: after correcting for the fact its name appears under three formatting variants in the underlying data, its combined total is 17,165 events - almost exactly double the next-largest manufacturer (Pfizer, 8,453).

1. Purpose and Scope

This report is standalone and does not depend on the causal or prescribing-linkage analyses previously reported. It treats F&B payment events purely as a measure of contact frequency between industry representatives and providers. This version extends the analysis window back to January 1, 2018 - the earliest full year commonly available - specifically to see whether patterns visible in the shorter 2023-2025 window are stable long-run features or artifacts of a short observation period.

Dollar value is deliberately set aside for this question: a \$12 coffee and a \$150 dinner program consume comparable provider time, so event count - not payment amount - is the correct unit for a time-burden analysis.

2. Data and Methodology

2.1 Source and Scope

Source: CMS Open Payments, national

Filters: Date_of_Payment between 2018-01-01 and 2025-12-31

Nature_of_Payment_or_Transfer_of_Value = 'FOOD AND BEVERAGE'

Covered_Recipient_Type restricted to physicians and non-physician practitioners (teaching hospitals excluded)

2.2 Comparison Groups

National = all qualifying US records.

NC / SC = Recipient_State = 'NC' / 'SC'

SRHS = SRHS_Provider = 'Y'

SRHS is a subset of the SC (and possibly NC) totals, not an independent population - at 1,273 of 33,294 SC providers (3.8%), the dilution effect on the SC baseline remains modest.

2.3 Two Caveats Specific To The Longer Window

First, 'providers' means providers with at least one qualifying F&B payment in the window, not all providers in that geography, as before. Second, SRHS Provider almost certainly reflects a current roster applied retroactively: a provider who joined SRHS in, say, 2023 would have their 2018-2022 payments (from wherever they worked previously) also tagged 'SRHS' under this flag. This means early-window SRHS figures may include some provider-years that do not reflect actual SRHS affiliation at the time, though there is no way to quantify this effect with the data and rosters available at the time of writing.

3. Results

3.1 Aggregate Comparison

Group	Providers	Events	Events/Provider (window)	Events/Provider/Year
National	1,691,078	88,946,219	52.60	6.58
NC	66,261	2,996,919	45.23	5.66
SC	33,294	1,810,032	54.37	6.80
SRHS	1,273	158,751	124.71	15.59

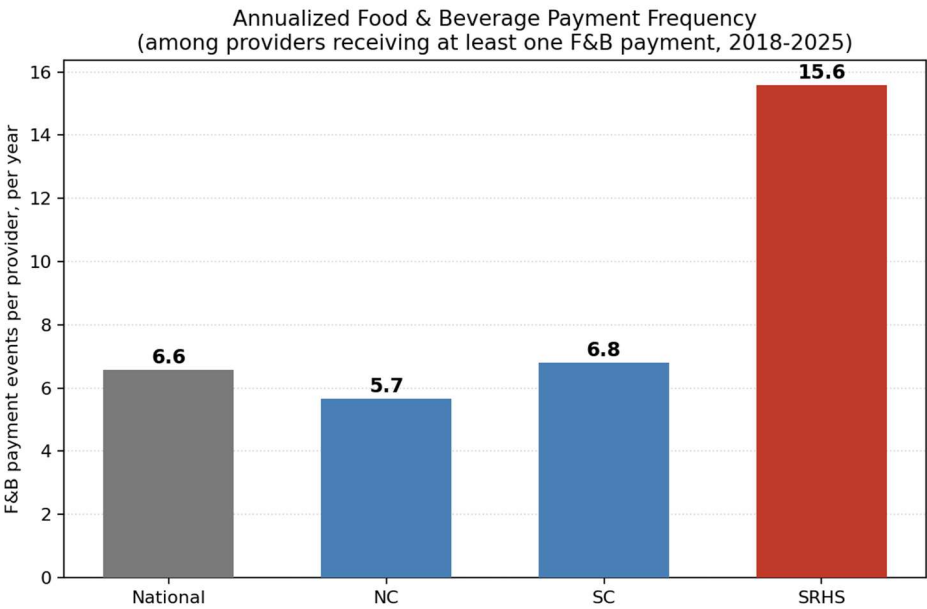


Figure 1. Annualized F&B events per provider (among providers with ≥ 1 F&B payment), full 2018-2025 window.

SRHS's annualized rate (15.6 events/provider/year) is 2.37x the national rate, 2.75x NC, and 2.29x SC - each of these ratios is larger than the equivalent figure from the shorter 2023-2025 window (2.23x/2.42x/2.05x), consistent with the widening trend documented in Section 3.5.

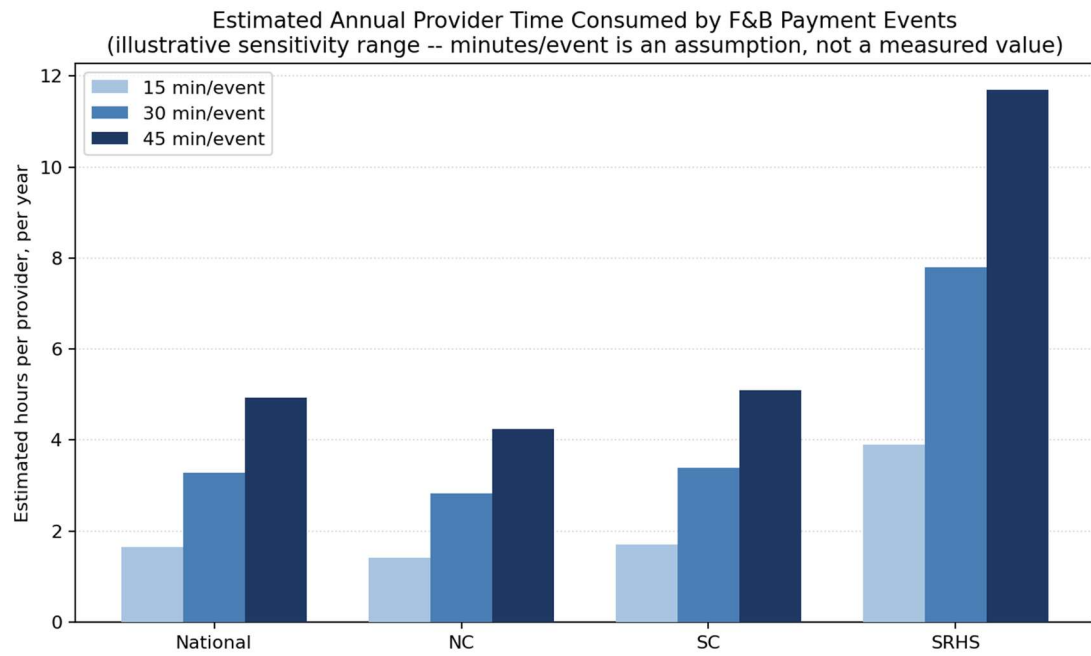


Figure 2. Estimated annual hours consumed, shown at three illustrative per-event duration assumptions (15/30/45 min). No field in this data measures actual minutes per encounter - this is a sensitivity range, not a measured quantity.

3.2 Distribution of Exposure - Not Just the Mean

As in the shorter window, the elevated SRHS average reflects a broadly shifted distribution, not a small number of extreme providers - and the shift is, if anything, more pronounced over the full eight years.

Group	P10	P25	P50 (median)	P75	P90	P95
National	1	2	8	35	129	236
NC	1	2	6	27	102	192
SC	1	2	8	35	129	247
SRHS	3	11	34	135	339	547

P99 is again omitted for National: 50,529 events for a single provider over eight years (roughly 17/day) is implausible as real contact and most likely a reporting/consolidation artifact. State and SRHS P99s (SRHS: 1,722) are more plausible and align with the top named provider in Section 3.6 (1,700-1,525 events). SRHS's median (34) is roughly 4-6x every peer group's median (6-8).

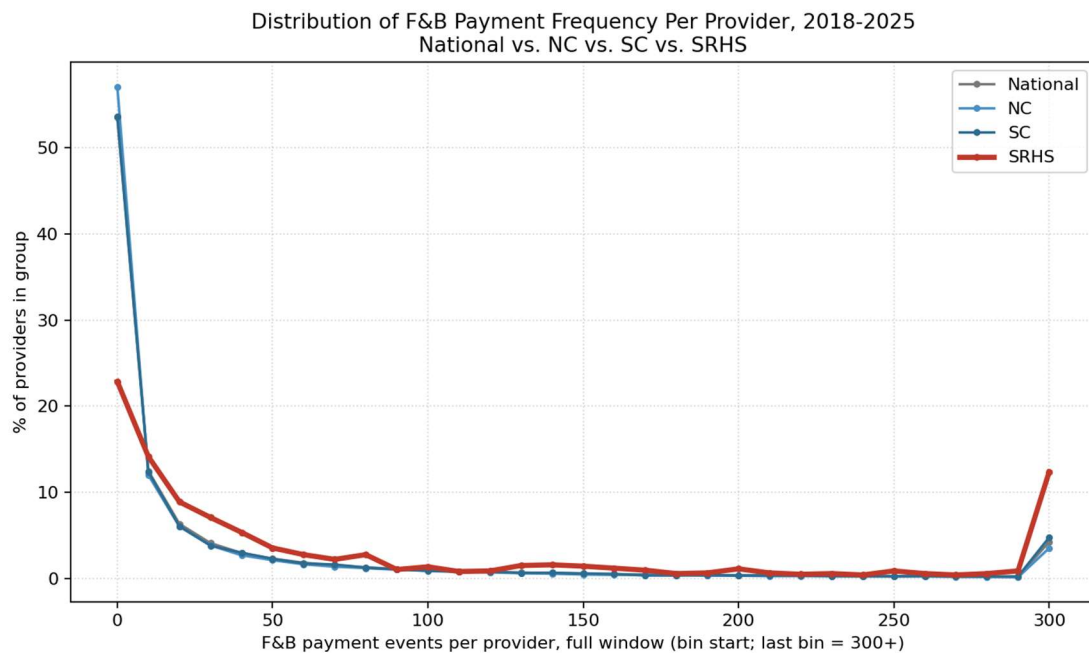


Figure 3. % of providers at each F&B-event-count level, 2018-2025. SRHS is a visibly shifted curve across the entire range.

22.9% of SRHS providers fall in the lowest bin (0-9 events over eight years) vs. 53.6-57.1% for the other three groups - under half the rate. At the other end, 12.3% of SRHS providers fall in the 300+ bin vs. 3.5-4.7% elsewhere - 2.6-3.5x higher.

3.3 Which Specialties Show Elevated Contact?

The longer window substantially improves this analysis: several specialties that had too few SRHS providers to trust in the 2023-2025-only version now clear a reasonable sample size threshold, because more provider-years of data accumulate more distinct individuals.

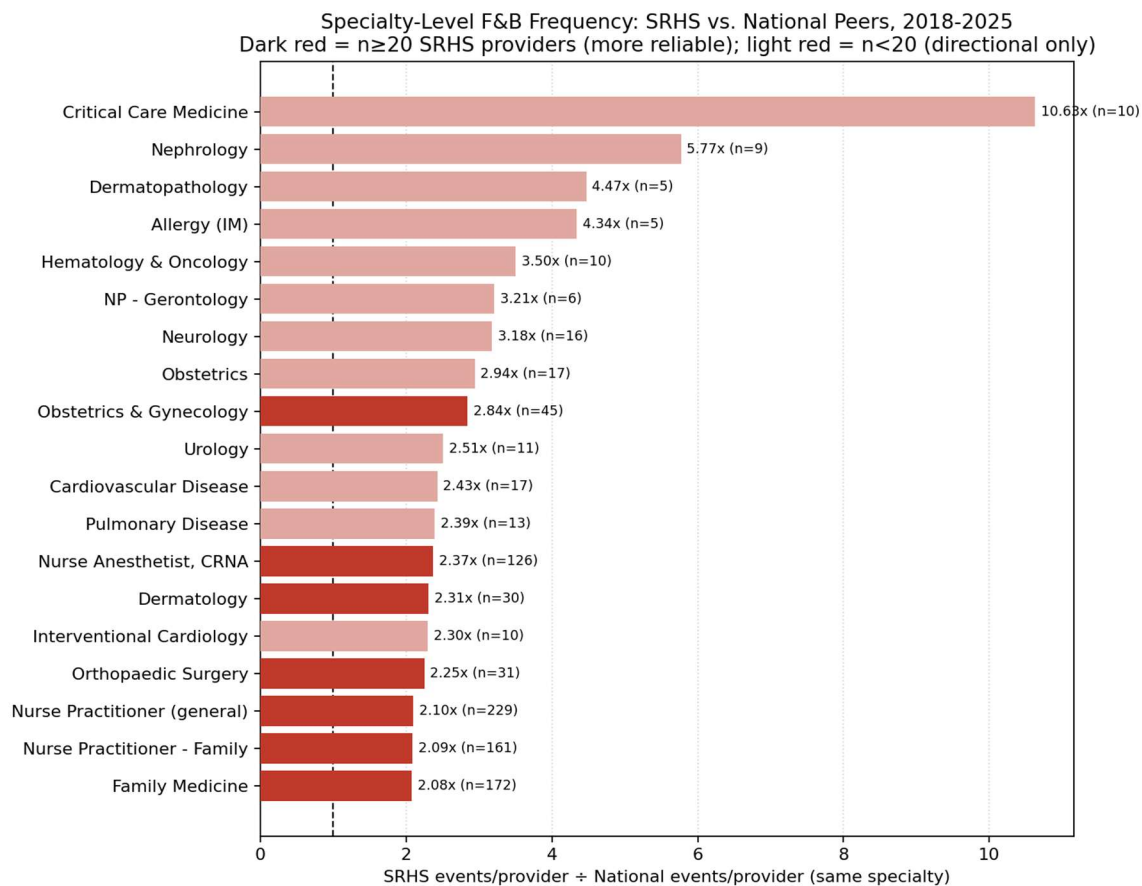


Figure 4. SRHS-to-national events-per-provider ratio by specialty, 2018-2025, restricted to specialties with ≥ 5 SRHS providers. Dark red = $n \geq 20$ (more reliable); light red = $n < 20$ (directional).

Seven specialties now combine a large, credible SRHS sample ($n \geq 20$) with a clearly elevated ratio:

- Obstetrics & Gynecology (2.84x, $n=45$)
- Nurse Anesthetist/CRNA (2.37x, $n=126$)
- Dermatology (2.31x, $n=30$)
- Orthopaedic Surgery (2.25x, $n=31$)
- Nurse Practitioner general (2.10x, $n=229$)
- Nurse Practitioner - Family (2.09x, $n=161$)
- Family Medicine (2.08x, $n=172$)

This is a materially stronger evidence base than the shorter window offered (only two qualifying specialties there).

Critical Care Medicine (10.63x), Nephrology (5.77x), Dermatopathology (4.47x), Allergy (4.34x), and Hematology & Oncology (3.50x) show the largest ratios of all, but on samples of 5-10 SRHS providers each - still directional flags rather than established findings, same as before.

3.4 Which Manufacturers Are Driving Interruptions?

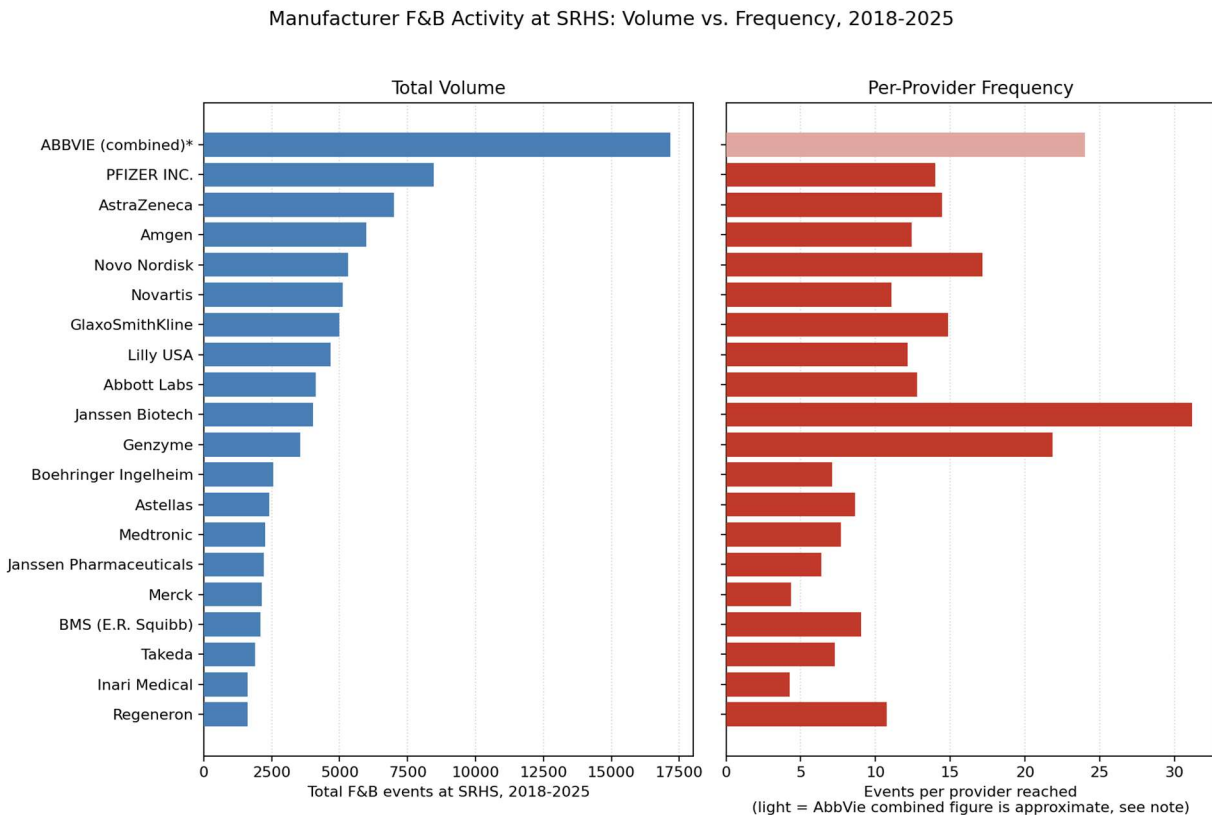


Figure 5. Left: total F&B events at SRHS by manufacturer, 2018-2025. Right: events per provider reached.

AbbVie's name appears under three formatting variants in the underlying data - "ABBVIE INC." (528 providers, 12,689 events), "AbbVie Inc." (314 providers, 3,271 events), and "AbbVie, Inc." (130 providers, 1,205 events) - almost certainly the same legal entity recorded inconsistently across years, not three distinct companies (compare to Janssen Biotech and Janssen Pharmaceuticals, which are genuinely separate registered entities and were kept separate). Summed, AbbVie's combined total is 17,165 events and \$276,608.64 - essentially double Pfizer, the next-largest manufacturer at 8,453 events. The combined per-provider frequency figure (24.0 events/provider, shown in lighter red) is an approximation: summing providers-reached across the three variants (528+314+130=972) would double-count any provider paid under more than one variant, so the true unique-provider count is somewhat lower and the true frequency somewhat higher than shown.

Janssen Biotech shows the single highest per-provider frequency of any manufacturer (31.2 events/provider across 129 providers) - a concentrated-relationship pattern distinct from AbbVie's broad-and-frequent one. Genzyme (21.8/provider, 162 providers) shows a similar concentrated pattern.

3.5 Is This Stable, Worsening, or Improving?

This is the section that changes most with the longer window. The 2023-2025-only version of this report concluded SRHS's elevated rate was a 'stable, persistent gap' with no drift - true as far as that window went, but that window started after the trend documented here had already begun.

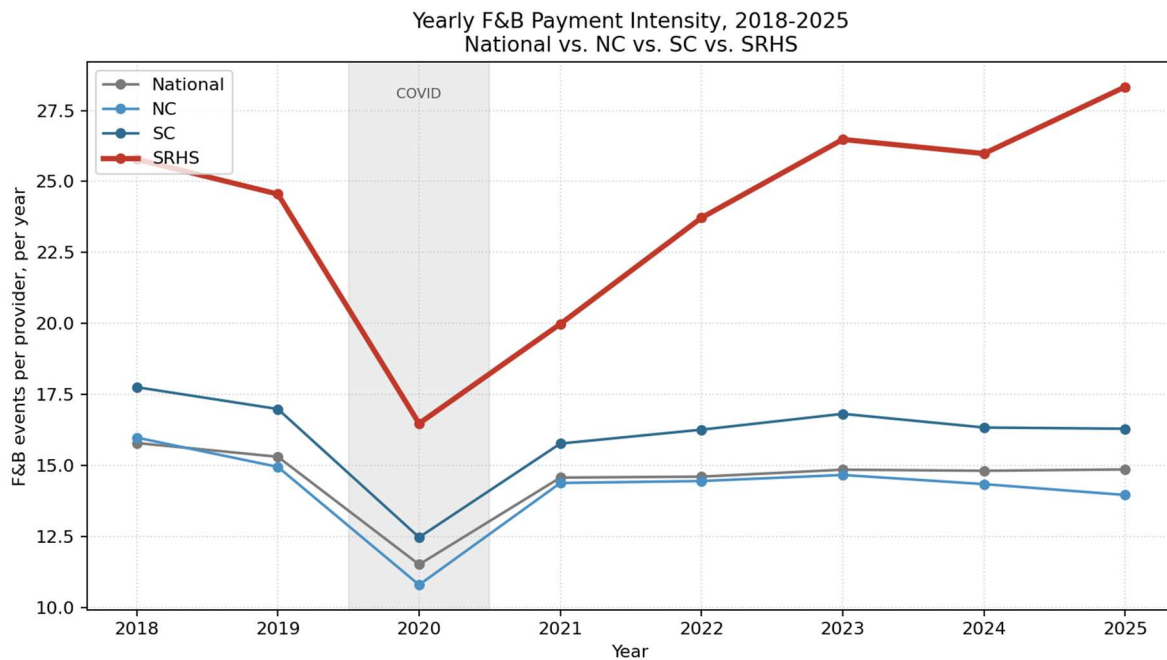


Figure 6. F&B events per provider, by year, 2018-2025. All four groups dip in 2020 (COVID-19); SRHS alone continues rising well past its 2018 level after recovering.

Year	SRHS ÷ National	SRHS ÷ SC
2018	1.63	1.45
2019	1.61	1.45
2020	1.43	1.32
2021	1.37	1.27
2022	1.62	1.46
2023	1.78	1.58
2024	1.75	1.59
2025	1.91	1.74

From 2018 to 2021, SRHS ran 1.3-1.6x national and regional peers - elevated, but with a narrower gap than the 2023-2025 window suggested, and the gap actually compressed slightly through the COVID period (2020-2021, likely reflecting a shared disruption to in-person rep visits across all groups). Beginning in 2022, the ratio has climbed every year, reaching 1.91x national and 1.74x SC in 2025 - both the highest values in the eight-year series. National, NC, and SC all plateaued within a point or two of their pre-COVID baseline once recovered; SRHS instead surpassed its own 2018 peak (25.8) by 2025 (28.3).

3.6 Who Exactly? The Highest-Frequency SRHS Providers

Grouping by NPI and name only (not specialty, which produced a split-record artifact in the prior version of this report) gives a clean, deduplicated top list over the full window.

Provider	Specialty	Events	Distinct Companies	Total \$
Carol Kooistra	Neurology	1,700	90	\$29,828.45
Matthew Miller	Dermatology	1,545	58	\$27,028.17
Amylynne Frankel	Dermatology/Dermatopathology	1,525	34	\$44,272.17
David Wilt	Nurse Practitioner	1,429	82	\$27,313.30
Bogdan Gheorghiu	Neurology	1,418	87	\$25,261.19
Joseph Boscia	Pulmonary Disease	1,146	57	\$26,978.75
Nancy Richmond	Nurse Practitioner - Family	1,107	41	\$18,663.67
Todd Schlesinger	Dermatology	1,080	55	\$56,369.08
Reeli Reinu	Physician Assistant	1,036	33	\$26,625.42
Brian Brown	Internal Medicine	1,018	53	\$34,225.05

Ten providers exceed 1,000 F&B events over the eight-year window - roughly one event every 2-3 days, sustained for years. As before, several of the highest-frequency providers (Kooistra at 90 distinct companies, Gheorghiu at 87, Wilt at 82) are paid by a very broad swath of the industry simultaneously, consistent with the 'hub provider' pattern documented elsewhere in this project. Dermatology again appears disproportionately (Miller, Frankel, Schlesinger - three of the top ten), consistent with Dermatology's elevated national baseline (Section 3.3) compounded further at SRHS specifically.

3.7 How Concentrated Is This Among a Few Providers?

Section 3.6 named individuals; this quantifies how much of the total SRHS F&B volume they - and the tiers below them - actually represent.

Top N NPIs	% of SRHS F&B-Paid Providers	Cumulative % of All SRHS F&B Events
5	0.39%	4.83%
10	0.79%	8.57%
25	1.96%	17.72%
50	3.93%	29.50%
100	7.86%	46.84%
200	15.71%	67.41%

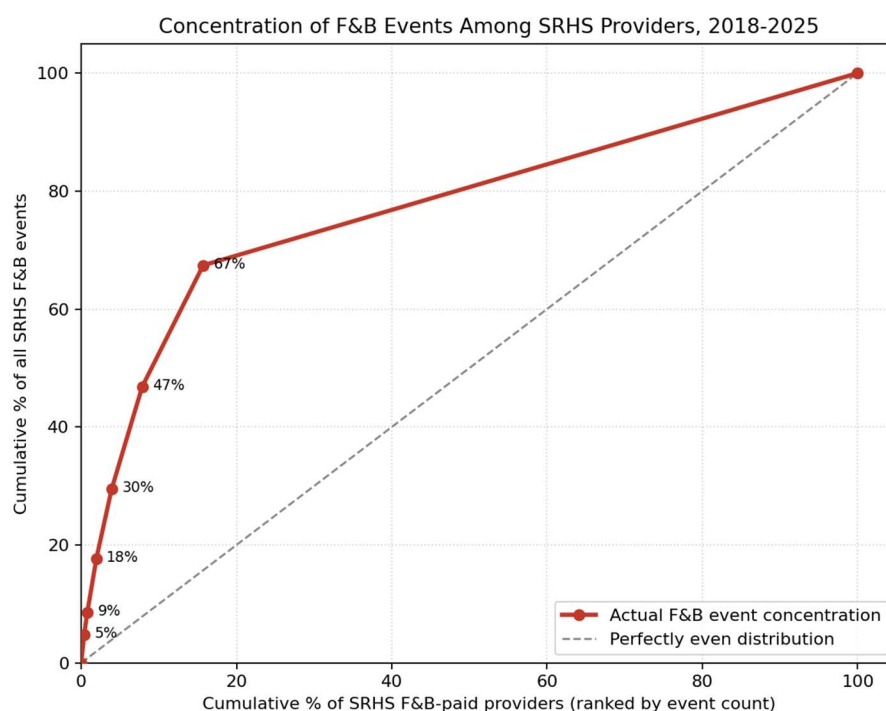


Figure 7. Cumulative share of SRHS F&B events by provider rank (1,273 total SRHS F&B-paid providers), against a perfectly even distribution for reference.

The concentration is sharp: the top 25 providers - under 2% of everyone who received any SRHS F&B payment - account for nearly 1 in 5 of all such events (17.72%). Widen the lens to the top 100 (7.86% of providers, still a small fraction) and it's nearly half of all events (46.84%).

This is the same shape as the manufacturer-side concentration finding elsewhere in this project (a small share of companies driving most payment volume) mirrored on the recipient side: F&B contact at SRHS is not spread evenly across the paid population, it is heavily concentrated in a few hundred individuals, with the most-contacted providers sitting at the very top of that curve.

4. Limitations

- The minutes-per-event figures in Section 3.1 are an illustrative assumption, not a measured quantity - no field in this data records actual encounter duration.
- 'Providers' throughout means providers with ≥ 1 qualifying F&B payment, not all providers in a geography.
- SRHS_Provider is a current-roster flag applied retroactively across the full 2018-2025 history; providers who joined SRHS partway through the window may have earlier, pre-affiliation payments counted as SRHS activity. This cannot be corrected with the fields available in this dataset and should be kept in mind.
- National P99 (50,529 events for one provider) is almost certainly a data artifact and was excluded from the percentile table.
- Specialty ratios for groups with fewer than 20 SRHS providers are sensitive to one or two high-frequency individuals and should be treated as directional, not conclusive.
- This report measures volume of contact, not causality or clinical impact - it does not establish that F&B frequency affects prescribing, referral patterns, or care quality.
- This report has no true peer-health-system benchmark - CMS Open Payments does not link individual providers to a multi-location employer/system, so National/NC/SC remain the only available comparison points.

5. Conclusion

Over the full eight years of available data, SRHS providers show a consistently elevated volume of Food & Beverage payment contact relative to national, North Carolina, and South Carolina peers. Unlike the shorter 2023-2025 window suggested, that gap has been widening since 2022 rather than holding steady, reaching its highest point in the series in 2025. The finding is broad-based across the distribution, not driven by a handful of outliers, and is now backed by credible sample sizes in seven specialties (led by Obstetrics & Gynecology, Nurse Anesthetists, Dermatology, Orthopaedic Surgery, and primary care roles).

A single manufacturer, AbbVie, accounts for a disproportionate and likely understated share of this volume once its name-variant records are combined; roughly double the next-largest manufacturer. F&B contact is also sharply concentrated on the recipient side: the top 25 providers, under 2% of everyone paid, account for nearly a fifth of all events.